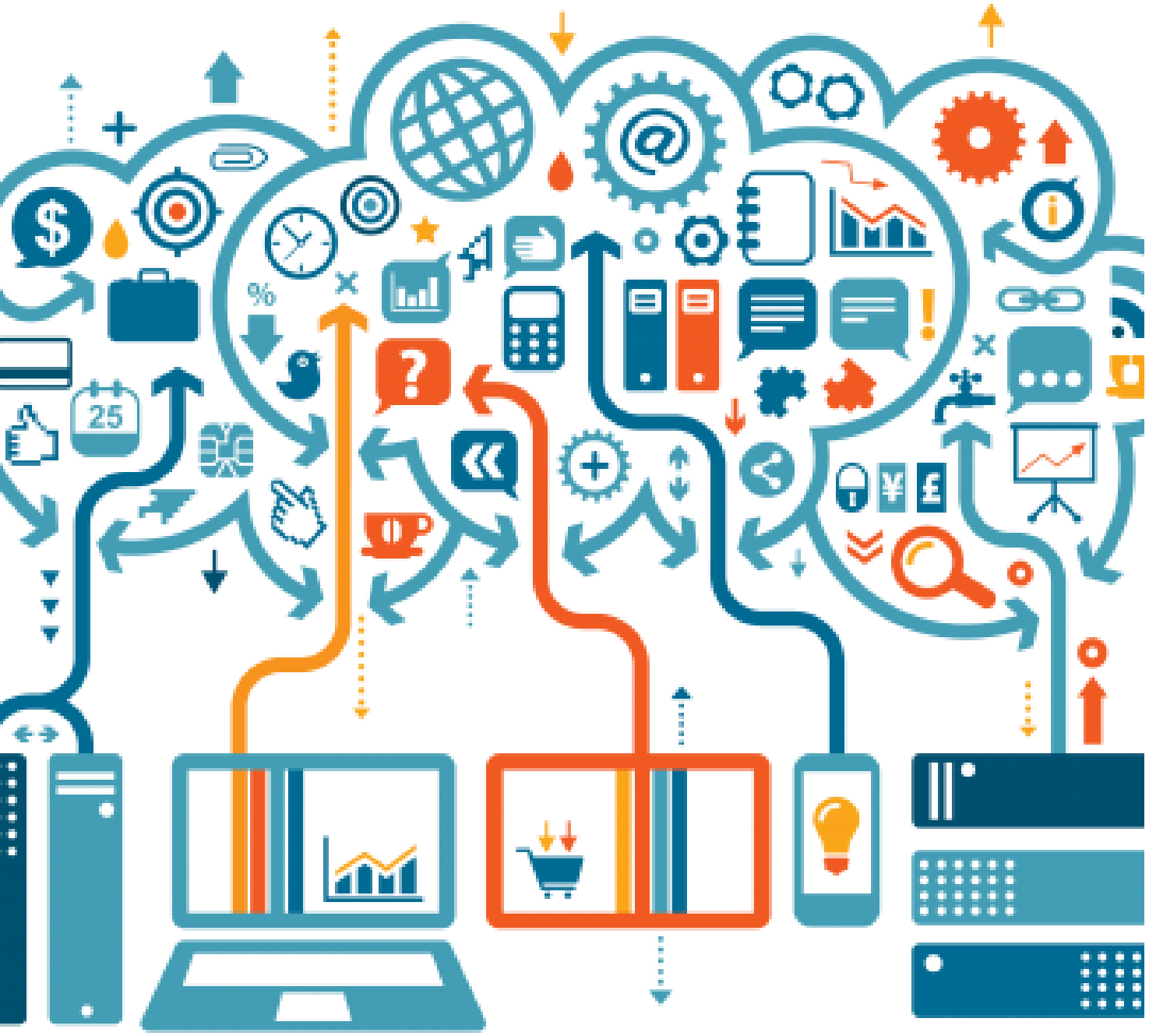
The (EU) regulation of health data sharing

Tjaša Petročnik, TILT, Tilburg Law School
t.petrocnik@tilburguniversity.edu



What is data (sharing) and why is it important?

Economic characteristics of data (see *Martens et al. 2020*)

Data is not an ordinary good

- ❑Non-rivalrous
- ❑Non-excludable
- ❑Zero marginal cost

Economic efficiencies in data use

- ❑Economies of scope
- ❑Economies of scale
- ❑Network effects

Why would we need to regulate data sharing?



Protecting fundamental rights



Addressing market failures



Facilitating innovation

What is health data?

Sensitivity and computational distance (*Malgieri and Comandé, 2017*)

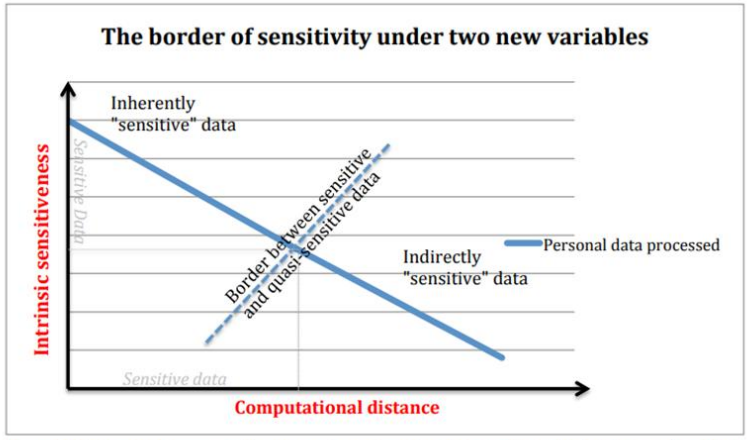
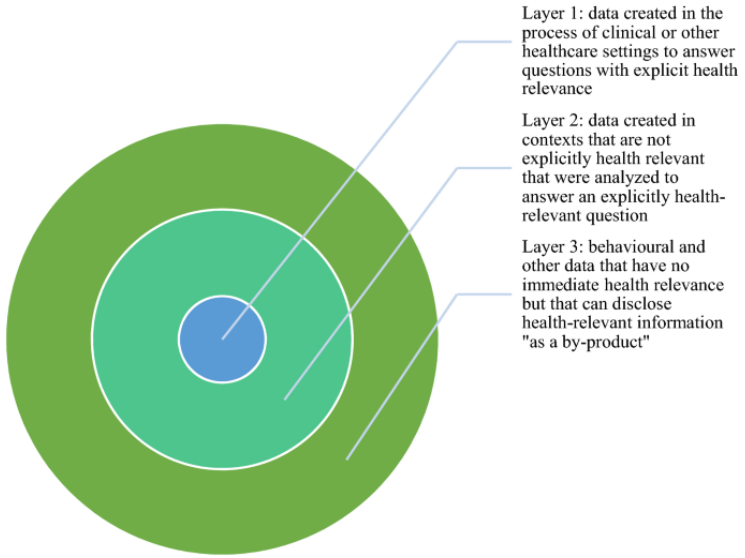


Figure 2. The border between sensitive data and quasi-sensitive data considering the intrinsic quality of data and computational distance at a certain historical period of technological development.

Purposes of use and relevance (*Fiske et al., 2022*)









The health data sharing imperative





The barriers to sharing health data

Medical privacy and confidentiality

Technical and organisational fragmentation

Data quality issues

Data as a parameter of competition

Public trust and political sensitivity

The regulation of healthcare in the EU

EU COMPETENCES

MS COMPETENCES

The regulation of health data sharing

Incentive alignment problems in healthcare

Insufficient (voluntary) data sharing

The GDPR

Article 4

Definitions

For the purposes of this Regulation:

(1) 'personal data' means any information relating to an identified or identifiable natural person ('data subject'); an identifiable natural person is one who can be identified, directly or indirectly, in particular by reference to an identifier such as a name, an identification number, location data, an online identifier or to one or more factors specific to the physical, physiological, genetic, mental, economic, cultural or social identity of that natural person;

(2) 'processing' means any operation or set of operations which is performed on personal data or on sets of personal data, whether or not by automated means, such as collection, recording, organisation, structuring, storage, adaptation or alteration, retrieval, consultation, use, disclosure by transmission, dissemination or otherwise making available, alignment or combination, restriction, erasure or destruction;

- Legal bases (Art. 6)
- Principles of data processing (Art. 5)
- Data subject rights (Ch. 3)
- Obligations for controllers and processors (Ch. 4)

Regulation (EU) 2016/679 of the European Parliament and of the Council of 27 April 2016 on the protection of natural persons with regard to the processing of personal data and on the free movement of such data, and repealing Directive 95/46/EC (General Data Protection Regulation) (Text with EEA relevance)

OJ L 119, 4.5.2016, pp. 1–88 (BG, ES, CS, DA, DE, ET, EL, EN, FR, GA, HR, IT, LV, LT, HU, MT, NL, PL, PT, RO, SK, SL, FI, SV)

The GDPR and health data

- Art. 4(15): ‘data concerning health’ means personal data related to the physical or mental health of a natural person, including the provision of health care services, which reveal information about their health status (present, past, future) (see also Rec. 35)

In summary, personal data are health data when:

1. The data are inherently/clearly medical data
2. The data are raw sensor data that can be used in itself or in combination with other data to draw a conclusion about the actual health status or health risk of a person
3. Conclusions are drawn about a person's health status or health risk (irrespective of whether these conclusions are accurate or inaccurate, legitimate or illegitimate, or otherwise adequate or inadequate)

ANNEX - health data in apps and devices

Article 9

Processing of special categories of personal data

1. Processing of personal data revealing racial or ethnic origin, political opinions, religious or philosophical beliefs, or trade union membership, and the processing of genetic data, biometric data for the purpose of uniquely identifying a natural person, data concerning health or data concerning a natural person's sex life or sexual orientation shall be prohibited.

2. Paragraph 1 shall not apply if one of the following applies:

- Higher level of protection for health data, but …
- 'Scientific research regime' provides for flexibility
  - Research exemption (Art. 9(2)(j)) in acc. with safeguards under Art. 89(1)
  - Relaxation of purpose and storage limitation and derogations to data subject rights if their exercise would hamper research aims
- States may maintain or introduce further conditions regarding health data processing (Art. 9(4))

European Strategy for Data

A common European data space, a single market for data



Data can flow within the EU and across sectors

Availability of high quality data to create and innovate

European rules and values are fully respected

Rules for access and use of data are fair, practical and clear & clear data governance mechanisms are in place

European Commission

Common European data spaces

Rich pool of data (varying degree of accessibility)

Free flow of data across sectors and countries

Full respect of GDPR

Horizontal framework for data governance and data access



- Technical tools for data pooling and sharing
- Standards & interoperability (technical, semantic)
- Sectoral Data Governance (contracts, licenses, access rights, usage rights)
- IT capacity, including cloud storage, processing and services

The DGA

Regulation (EU) 2022/868 of the European Parliament and of the Council of 30 May 2022 on European data governance and amending Regulation (EU) 2018/1724 (Data Governance Act) (Text with EEA relevance)

PE/85/2021/REV/1

OJ L 152, 3.6.2022, pp. 1–44 (BG, ES, CS, DA, DE, ET, EL, EN, FR, GA, HR, IT, LV, LT, HU, MT, NL, PL, PT, RO, SK, SL, FI, SV)

- Reuse of certain public sector data (Ch. 2)
  - Types of data (Art. 3)
  - Conditions for reuse (Art. 5), but …
  - No obligation on public sector bodies to allow the reuse of data (Art. 1(2))
- Rules for data intermediaries (Ch. 3)

> (11) 'data intermediation service' means a service which aims to establish commercial relationships for the purposes of data sharing between an undetermined number of data subjects and data holders on the one hand and data users on the other, through technical, legal or other means, including for the purpose of exercising the rights of data subjects in relation to personal data, excluding at least the following:

- Data altruism (Ch. 4)

> (16) 'data altruism' means the voluntary sharing of data on the basis of the consent of data subjects to process personal data pertaining to them, or permissions of data holders to allow the use of their non-personal data without seeking or receiving a reward that goes beyond compensation related to the costs that they incur where they make their data available for objectives of general interest as provided for in national law, where applicable, such as healthcare, combating climate change, improving mobility, facilitating the development, production and dissemination of official statistics, improving the provision of public services, public policy making or scientific research purposes in the general interest;

The DA

Regulation (EU) 2023/2854 of the European Parliament and of the Council of 13 December 2023 on harmonised rules on fair access to and use of data and amending Regulation (EU) 2017/2394 and Directive (EU) 2020/1828 (Data Act) (Text with EEA relevance)

PE/49/2023/REV/1

OJ L, 2023/2854, 22.12.2023, ELI: http://data.europa.eu/eli/reg/2023/2854/oj (BG, ES, CS, DA, DE, ET, EL, EN, FR, GA, HR, IT, LV, LT, HU, MT, NL, PL, PT, RO, SK, SL, FI, SV)

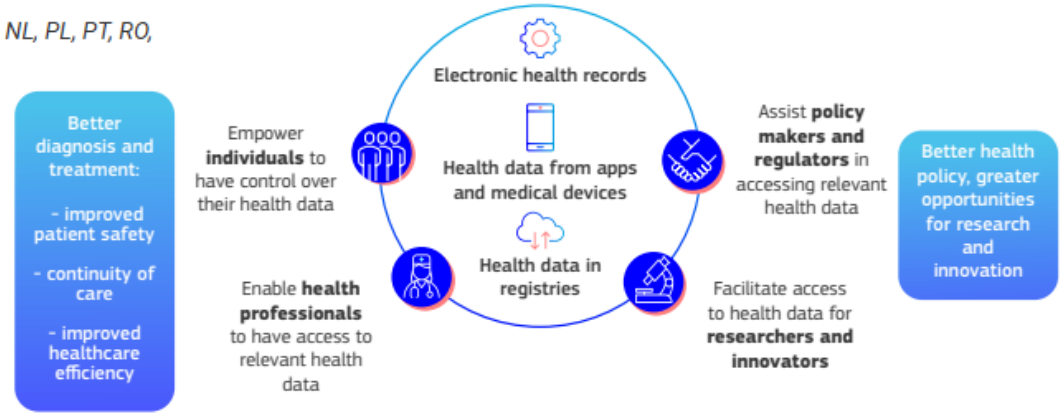
- Rules for B2C and B2B IoT data sharing
  - Art. 3: Connected products and related services have to be designed in such a way that data is by default, easily, securely, comprehensively, free of charge, and in a structured, commonly used and machine-readable format directly accessible to the user
  - Art. 4: data holders shall make readily available data, as well as the relevant metadata necessary to interpret and use those data, accessible to the user without undue delay
  - Art. 5: data holders shall make readily available data, as well as the relevant metadata necessary to interpret and use those data, available to a third party upon request of the user, but …
    - Restrictions: no gatekeepers (Art. 5(3)) and not to develop competing products (Art. 6)
  - Fair, reasonable, and non-discriminatory arrangements (Art. 8) and compensation for making data available (Art. 9)
- Rules for B2G data sharing based on exceptional need (Ch. 5)

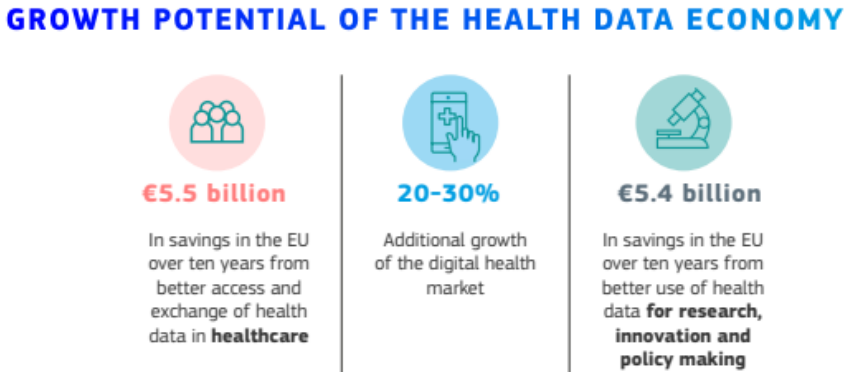
Regulation (EU) 2025/327 of the European Parliament and of the Council of 11 February 2025 on the European Health Data Space and amending Directive 2011/24/EU and Regulation (EU) 2024/2847 (Text with EEA relevance)

PE/76/2024/REV/1

OJ L, 2025/327, 5.3.2025, ELI: http://data.europa.eu/eli/reg/2025/327/oj (BG, ES, CS, DA, DE, ET, EL, EN, FR, GA, HR, IT, LV, LT, HU, MT, NL, PL, PT, RO, SK, SL, FI, SV)

The EHDS





- Rationale: to put forward a harmonised regime for secure and free movement of health data in the EU (see Rec. 110)
  - Empower natural persons with control over their health data
  - Foster internal makret for digital helath
  - Ensure consistent and efficient framework for reuse of health data
- The EHDSR thus establishes the common rules, rights, standards, infrastructures, and governance to facilitate access to health data (= personal or non-personal electronic health data (Rec. 6, Art. 2(2)(c))) for primary and secondary use (Art. 1)

Primary use of health data

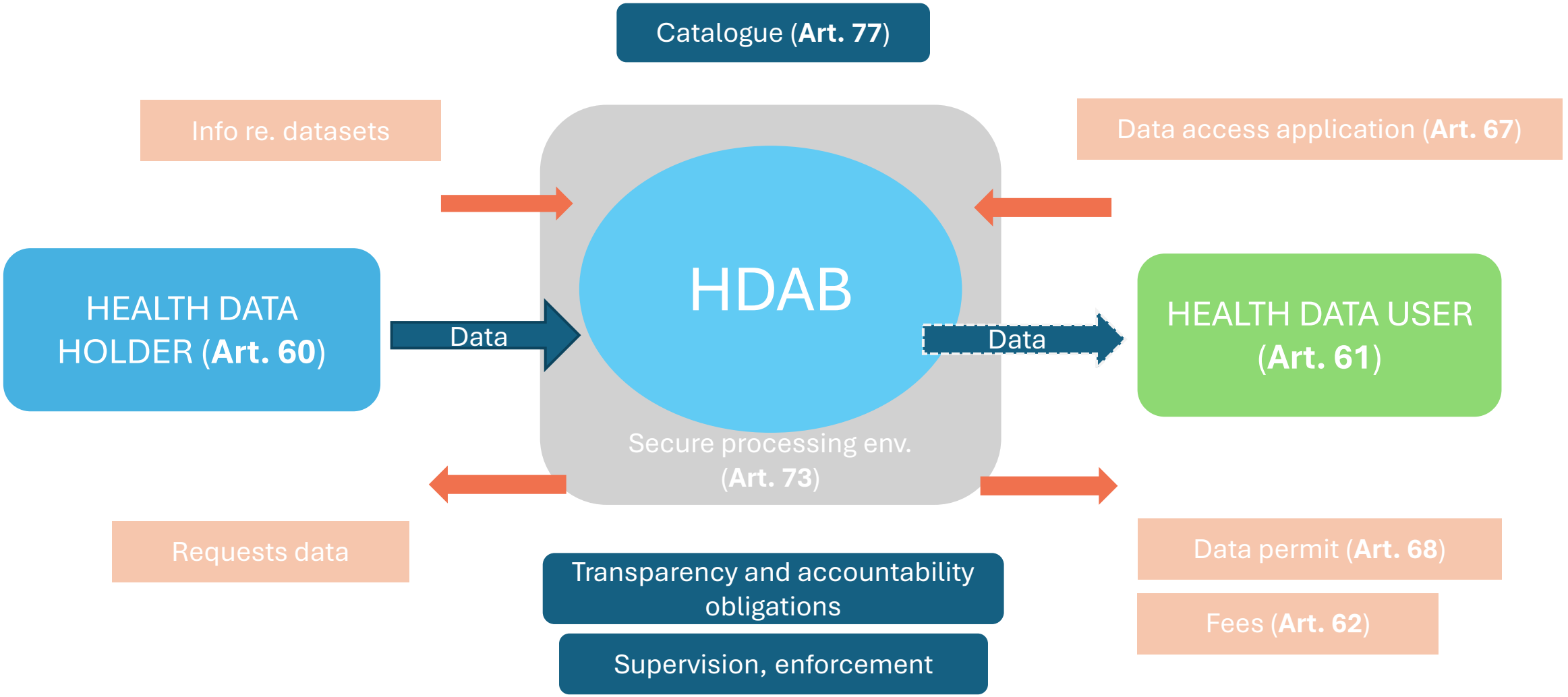


- Processing of health data for the provision of healthcare (Art. 2(2)(d))
- Regulatory mechanisms:
  - Priority categories of health data (Art. 14)
  - European electronic health record exchange format (Art. 15) and interoperability and other requirements for EHRs (Ch. 3)
  - Rights of natural persons (Art. 3-10) and the right of health professionals to access personal health data in EHRs (Art. 11)
    - To facilitate the exercising of these rights, Member States should establish electronic health data access services (Art. 4) and health professional access services (Art. 12)
  - Cross-border infrastructure MyHealth@EU (Art. 23)

Secondary use of health data

- Processing of health data for purposes other than initial purposes for which they were collected or produced (Art. 2(2)(e))
  - Research, innovation, policy-making, education, patient safety, regulatory activities or personalised medicine
  - Access to data for secondary use should contribute to the general interest of society (Rec. 61)
- Regulatory mechanisms:
  - Minimum categories of health data for secondary use (Art. 51)
    - Data minimisation and purpose limitation (Art. 66)
  - Delineation of allowed and prohibited purposes (Art. 53-4)
  - Administrative procedure (a permit scheme) to facilitate secondary use (Art. 55)
  - Infrastructure HealthData@EU (Art. 75)




Catalogue (Art. 77)
Info re. datasets
Data access application (Art. 67)
HDAB
HEALTH DATA HOLDER (Art. 60)
Data
Data
HEALTH DATA USER (Art. 61)
Secure processing env. (Art. 73)
Requests data
Transparency and accountability obligations
Supervision, enforcement
Data permit (Art. 68)
Fees (Art. 62)

Sources and further reading

- Amelia Fiske et al., ‘Value-creation in the health data domain: a typology of what health data help us do’ (2022) 18 Biosocieties 473
- Gianclaudio Malgieri & Giovanni Comandé, ‘Sensitive-by-distance: quasi-health data in the algorithmic era’ (2017) 26 Information & Communications Technology Law 229
- Bertin Martens et al., ‘Business-to-business data sharing: An economic and legal analysis’ (2020) JRC Digital Economy Working Paper Series, no. 05, vol. 2020 <https://ec.europa.eu/jrc/sites/jrcsh/files/jrc121336.pdf>
- Elizabeth Pisania & Carla Abou Zahr, ‘Sharing health data: good intentions are not enough’ (2010) 88 Bulletin of the World Health Organization 462
- WP29 Letter – Annex: Health data in apps and devices (2015) <https://ec.europa.eu/justice/article-29/documentation/other-document/files/2015/20150205_letter_art29wp_ec_health_data_after_plenary_annex_en.pdf>